RESPIRATORY CASE

↓

DECODING HISTORY & EXAMINATION

40yr old male

C/C → (a) Fever ± chills x 3d
(b) Productive cough x 2d
(c) SOB x 2d

Fever, undocumented, ass. ± chills, daily spikes ⊕.

Productive cough → rusty brown
non-foul smelling
no ass. ± posture

SOB → preceded by cough, more on lying on left side
Not ass. ± PND, Orthopnea
Not prev H/O SOB

Not ass. ± odynophagia/sore-throat/hoarseness/stridor/wheeze

No h/o preceding coryza ⊕.

No h/o bluish discolorn, altered sensorium.

Personal → Chronic alcoholic & smoker.

O/E → GPE → Pallor ⊕
Icterus ⊖
Cyanosis ⊖
Clubbing ⊖
Lymphadenopathy ⊖
Edema ⊖

No conjuctival injection.

No TB stigmata ⊕.

Respiratory exn → O/I → Nasal flaring ⊕
use of accesory muscles ⊕
Trachea in midline
Chest movements → ↓ on Rt. side
Apex impulse → 5th IC space, just medical to MCL.

O/Palpⁿ → Diameters (N) (A/P, T)
Expansion ↓ on Rt. side

VF → ↑ in Rt. infra-axillary & sub-scapular areas

O/Percussⁿ → Woody dull note (+) in Rt. infraaxillary & subscapular areas

O/Auscⁿ → Bronchial breath sounds (+)

Vocal Resonance ↑ in Rt. infra-axillary & subscapular area.

Whispering Pectoriloquy (+)

Egophony (+) (E → A)

pleural / in cough ⊖ → dry

40yr old male

Fever + productive + SOB → infected airway d/o

HIGH GRADE FEVER

RUSTY BROWN SPUTUM

C/C → (a) Fever ± chills x 3d
(b) Productive cough x 2d
(c) SOB x 2d

Fever → infected, undocumented, ass. ± chills → high grade?, daily spikes ⊕.

infected d/o airway

DDx - Pharyngitis, Laryngitis, Bronchitis | Pneumonia, Lung Abscess, Bxis, Acute ex. of COPD

Productive cough → airway d/o

pleural cavity d/o infers

rusty brown (S. pneumonia)

non-foul smelling → lung abscess, Bxis

no ass. ± posture → lung abscess

SOB → Resp.

preceded by cough, more on lying on left side → Rt. sided pathology

Not ass. ± PND, Orthopnea [? CVS cause]

Resp. / CVS

Not prev H/O SOB → ✗ Bxis/COPD

Not ass. ± odynophagia / sore-throat / hoarseness / stridor / wheeze

? Pharyngitis ? Pharyngitis ? croup bronchitis?

DDx → Bronchitis, Pneumonia

No h/o preceding coryza ⊕. (prev. URTI → S. aureus)

No h/o bluish discolorn, altered sensorium. (Type I RF?)

Personal → Chronic alcoholic & smoker. (RFs for Strep) → Bronchitis, Consolidation

O/E → GPE →

Pallor ⊕ (? Chronic alcoholic ? Poor nutrition)

No conjuctival injection. (RF)

Cor pulmonale

ATT induced

Icterus ⊖

Cyanosis ⊖ → Type I

No TB stigmata ⊕.

Clubbing ⊖ → ⊖ COPD, TB; ⊕ Lung abscess, Bxis

Lymphadenopathy ⊖ → TB

Edema ⊖ (Bxis, Cor pulm., Type II RFs)

Respiratory exn → O/I → Nasal flaring ⊕, use of accessory muscles ⊕) distress

Trachea in midline (mild pleural cavity / ± in lung)

Chest movements → ↓ on Rt. side → Rt. side pathology

Apex impulse → 5th IC space, just medial to MCL.

C

O/ Palpⁿ → Diameters (N) (A/P, T) (Less likely to be chronic)
Expansion ↓ on Rt. side → Rt. sided

VF → ↑ in Rt. infra-axillary & sub-scapular areas
(↑ fluid in lung)**

O/ Percussⁿ → Woody dull note (+) in Rt. infraaxillary & subscapular areas

↑ fluid → cavity → collapse

O/ Auscⁿ → Bronchial breath sounds (+)

Vocal Resonance ↑ in Rt. infra-axillary & subscapular areas.

↑ fluid
- Whispering Pectoriloquy (+)
- Egophony (+) (E → A)

Infective pathology →
- ? Consolidation (R.)
  - productive, rusty
  - non-, pasture.
  - O/E → VF (+)
    - VR (+)
    - BS (+), WP, (E)
    - wood → Rt. side or,
- ? S. pneumonia
- Type I RF (-) → ↓ anosis